

SELADIN[®] TABLET

Presentation(s):

Seladin Tablet 250mg

Each tablet contains:

Naproxen 250mg

Seladin Tablet 500mg

Each tablet contains:

Naproxen 500mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetic):

1. Naproxen is a non-steroidal anti-inflammatory agent with analgesic and antipyretic activity. The exact mechanism of action have not been clearly established, but many appears to be associated principally with the inhibition of prostaglandin synthesis. It inhibits the synthesis of prostaglandin in body tissues by inhibiting cyclooxygenase, an enzyme that catalyzes the formation of prostaglandin precursors (endoperoxides) from arachidonic acid.
2. Improvement in patients treated for rheumatoid arthritis have been demonstrated by a reduction in joint swelling, a reduction in pain, a reduction in duration of morning stiffness, a reduction in disease activity as assessed by both the investigator and patients, and by increased mobility as demonstrated by an increase in walking time.
3. Naproxen has generally been better tolerated than aspirin or indomethacin at usual dosages. Because of its relatively long plasma half-life, it can be given twice daily, and there is more evidence that once daily dosage is effective in rheumatoid arthritis.
4. Naproxen is rapidly and completely absorbed from the gastrointestinal tract. After administration of naproxen, peak plasma levels of naproxen anion are attained in 2 ~ 4 hours, with steady-state conditions normally achieved after 4 ~ 5 doses. The mean biological half-life of the anion in human is approximately 13 hours, and at therapeutic levels it is greater than 99% albumin bound. Approximately 95% of the dose is excreted in the urine, primarily as naproxen, 6-O-desmethyl naproxen or their conjugates. The rate of excretion has been found to coincide closely with the rate of drug disappearance from the plasma. The drug does not induce metabolizing enzymes.

Indication(s):

For the relief of pain and inflammation in rheumatic disorders, acute gout and other musculoskeletal disorders.

Dosage and Administration:

In rheumatic disorders, the usual dose is 250mg twice daily, adjusted to 0.5g to 1g daily in 2 divided doses. A dose of 10mg/kg/day in 2 divided doses has been used in children over 5 years with juvenile rheumatoid arthritis. In acute gout, initial dose is 750mg followed by 250mg every 8 hours. In acute musculoskeletal disorders, 500mg initially, then 250mg every 6-8 hours as required.

After assessing the risk / benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Route of Administration:

Oral

Contraindication(s):

1. Known hypersensitivity to this product, aspirin or other NSAIDs.
2. Peptic ulceration: Naproxen causes a slight increase in fecal occult blood loss but it is relatively infrequently associated with peptic ulceration. It should not be used in patients with active peptic ulceration.
3. Severe renal impairment: the fall in glomerular blood flow induced by naproxen and other non-steroidal anti-inflammatory drug may precipitate irreversible renal failure in patients with pre-existing severe impairment of renal function.

Side Effect(s) / Adverse Reaction(s):

1. Gastrointestinal: The most frequent complaints are related to gastrointestinal tract. They are: constipation, heartburn, abdominal pain, nausea, dyspepsia, diarrhea, stomatitis.
2. Central nervous system: Headache, dizziness, drowsiness, lightheadedness, vertigo.
3. Dermatologic: Itching (pruritus), skin eruptions, ecchymoses, sweating, purpura.
4. Special senses: Tinnitus, hearing and visual disturbances.
5. Cardiovascular: Edema, dyspnea, palpitations.
6. Hematology: Blood disorders and occasional bleeding and ulceration.
7. General: Thirst

Precaution(s) / Warning(s):

1. Naproxen should be given with care to patients with bleeding disorders, cardiovascular disease, renal failure, and in those who are receiving coumarin anticoagulant.
2. All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occurs in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.
3. Use in pregnancy and lactation:
Naproxen causes a delay in parturition in animals and has been associated with premature closure of the ductus arteriosus and severe pulmonary hypertension in infant born to mothers taking naproxen. Naproxen is excreted in breast milk in small amount.
Because the formations of the organs in the newborn and neonate are not yet complete, risk should be weighed against benefit before the product is given to pregnant women and nursing mother (FDA Pregnancy category B).
4. Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.
There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.
5. NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking anti-hypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

6. Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.
7. NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reaction and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.
8. Short-term controlled studies failed to show that taking the drug significantly affects prothrombin times when administered to individuals on coumarin-type anticoagulants. Caution is advised, nonetheless, since interactions with this class have been seen with other non-steroidal agents.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Drug interactions:

1. Probenecid may inhibit both the renal excretion and hepatic metabolism of naproxen, result in increased plasma concentration of Naproxen.
2. Naproxen can enhance the effect of sulfonylurea, sulfonamide and hydantoin. Patients receiving naproxen with these drugs concurrently should be observed for signs of toxicity to these drugs.
3. Naproxen and other non-steroidal anti-inflammatory can antagonize the antihypertensive effect of propranolol and other beta-blockers.
4. Like other non-steroidal anti-inflammatory agents, naproxen may temporarily inhibit platelet aggregation when given in high doses, patients receiving naproxen and coumarin anti-coagulant at the same time may increase risk of haemorrhage.
5. Naproxen may raise plasma concentration of lithium when given together.
6. Caution is called for if non-steroidal anti-inflammatory agents are administered <24 hours before or after treatment with methotrexate, since methotrexate plasma level may rise and the toxicity of methotrexate be increased.

Symptoms and Treatment for Overdosage, and Antidote(s):

In overdosage the stomach should be emptied and activated charcoal given to try to reduce absorption.

Shelf-life:

3 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

Seladin Tablet 250mg

A slight yellow color tablet, one side impressed with a score.

Plastic bottle of 500's, 1000's and 1500's (for export only).

Blister packing of 10's x 10, 10's x 50 and 10's x 100.

Seladin Tablet 500mg

A slight yellow color tablet, one side impressed with a score, the other side with a "45" mark.



Plastic bottle of 1000's and 1500's (for export only).

Blister packing of 10's x 3, 10's x 10 and 10's x 50.



Manufacturer and Product Registration Holder:
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